

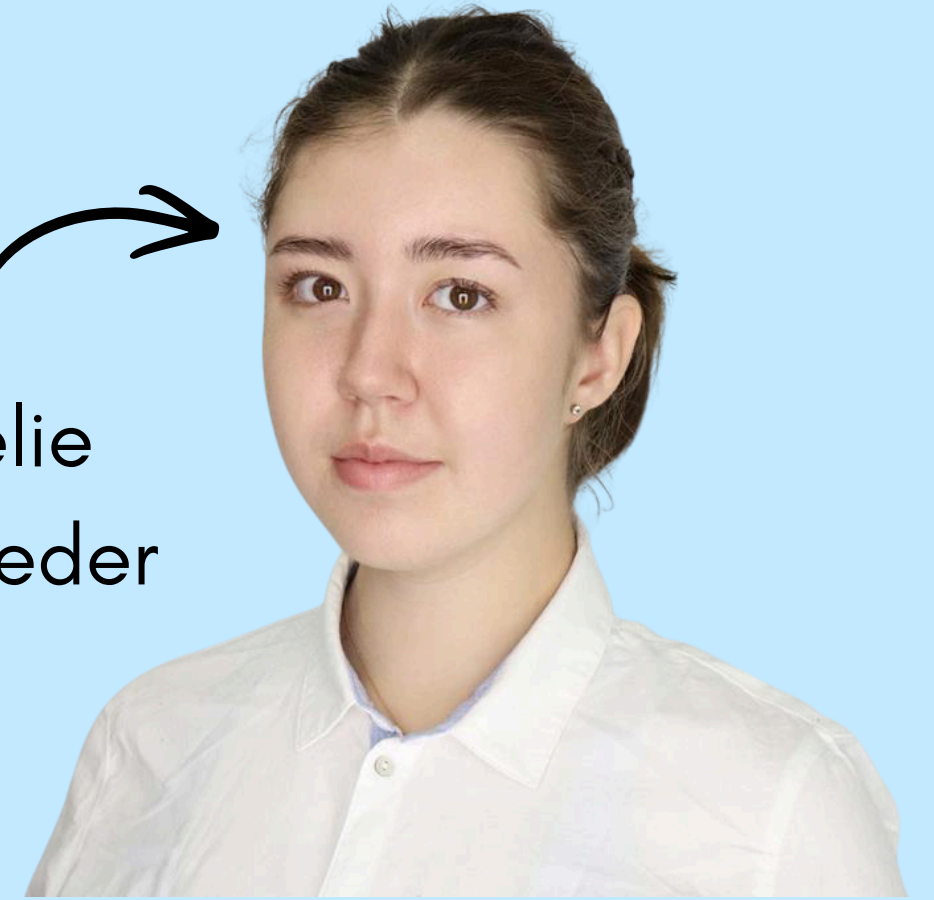
TEAM HAC



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ISSUES

- How can we transparently combine predicted outcomes and patient preferences to support shared decision making in clinical practice?

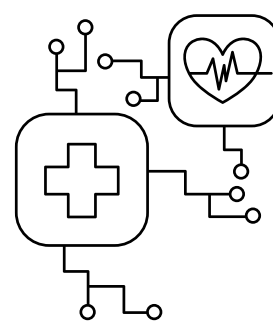


DATA

- Preferences vary: Patients do not allocate priorities consistently, so normalization is required.
- Outcomes differ: Pain, function, time, and cost use different scales and must be aligned before combining.
- Decisions are often borderline: Treatment choice depends on individual effects and patient values, supporting shared decision making.

METHOD

- Compare fairly: Convert different outcomes (pain, function, time, cost) into a common value so trade-offs are comparable.
- Personalize decisions: Combine patient-specific treatment effects with what each patient values most.
- Show uncertainty: Flag borderline and preference-sensitive cases to support shared decision making, not automatic rules.

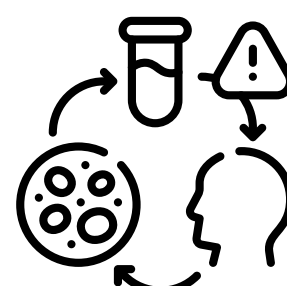


METHOD

- Mostly conservative: ~72% favor non-surgical care.
- Often borderline: Many decisions need shared decision making.
- Few clear cases: High-confidence recommendations are rare but decisive.

CLINICAL USAGE

- When: During treatment discussion.
- What: Patient-specific comparison with recommendation and confidence.
- How: Review trade-offs; use shared decision-making for borderline cases.



TAKEHOME/ RELEVANCE

- Combines predicted outcomes with patient values on a common scale.
- Makes trade-offs transparent and flags preference-sensitive cases.
- Supports—not replaces—clinical judgment.